

Spec. Taken

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle): <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials: <u>RB</u> Date Reviewed: <u>7/10/12</u>		

7/14/12

INTERPRETATION AND DIAGNOSIS:

1. PARATHYROID (BIOPSY): LYMPHOID AND FIBROADIPOSE TISSUE. NO PARATHYROID IDENTIFIED.

2. TOTAL THYROID AND CENTRAL NECK DISSECTION LEVEL 6 (THYROIDECTOMY AND CENTRAL NECK DISSECTION):

SPECIMEN TYPE:

Total thyroidectomy.

ICD-O-3
carcinoma, papillary, thyroid
8240/3
Site: thyroid, NOS C73.9
7-18-12 RO

TUMOR SITE:

Isthmus, left lobe.

HISTOLOGIC TYPE:

Papillary carcinoma with focal solid areas.

TUMOR SIZE:

Greatest dimension: 2.8cm.

UUID:32BD1BDE-E814-4CEC-8986-F3C60C43FCD4
TCGA-ET-A40T-01A-PR

Redacted

FOCALITY:

Unifocal.



LYMPH NODES:

Small foci of metastatic carcinoma in 3 of 15 lymph nodes. See note.

EXTENT OF INVASION (Edition AJCC):

PRIMARY TUMOR:

pT2: Tumor >2cm, but <=4cm, limited to thyroid.

REGIONAL LYMPH NODES:

pN1A: regional lymph node metastasis, See note.

MARGINS:

Margins are uninvolved. Distance of invasive carcinoma from nearest margin: <1mm. Specify margin: anterior and deep.

VENOUS/LYMPHATIC INVASION:

Present.

ADDITIONAL PATHOLOGIC FINDINGS:

Thyroiditis (lymphocytic).

NOTE: Psammomatous calcifications are present in 3 of 15 lymph nodes, and levels and immunostains for cytokeratin highlight individual metastatic carcinoma cells in association with these cells. Thyroglobulin is noncontributory (high background) while TTF-1 labels a few of these cells